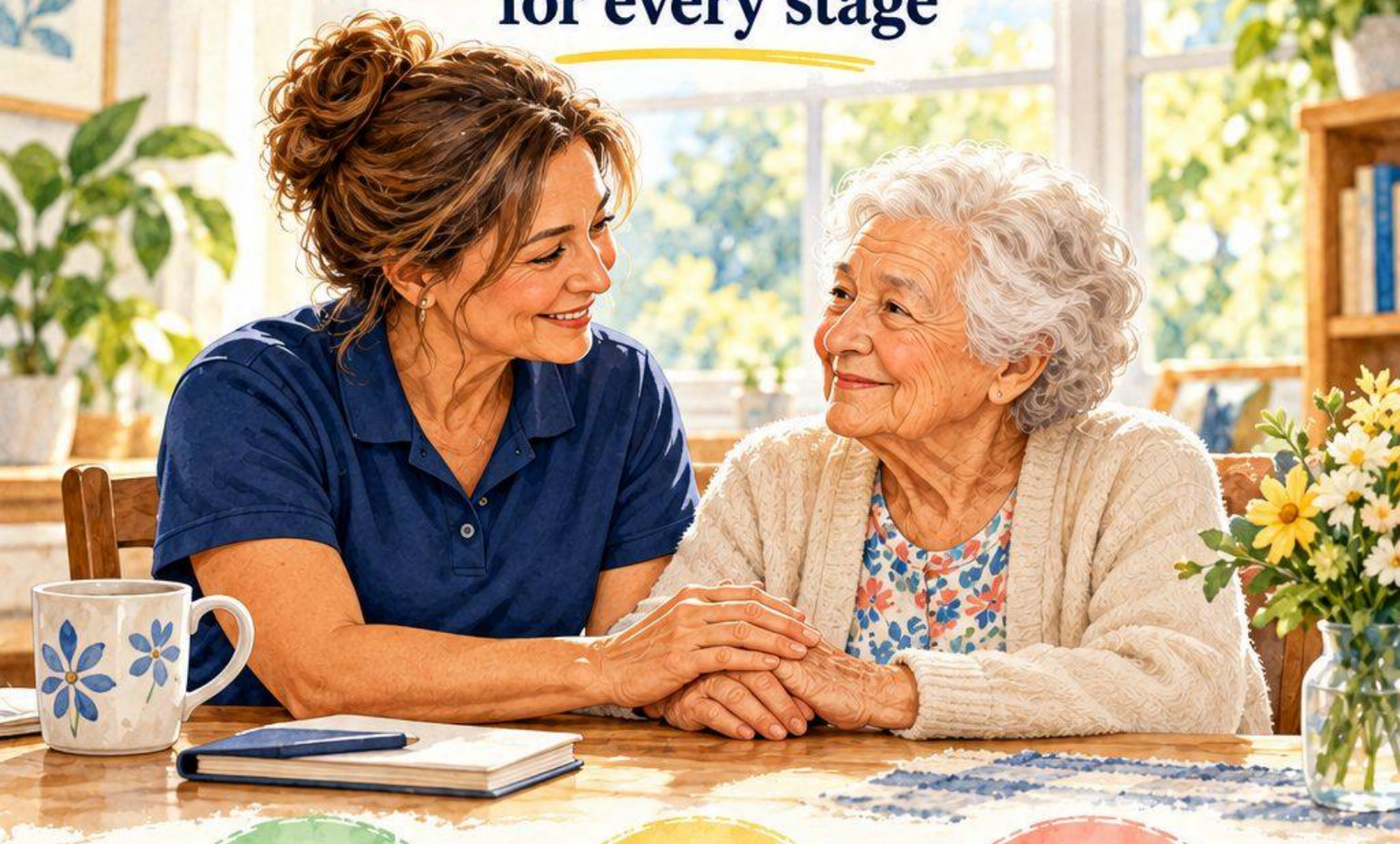
BRIGHTER DAYS

Alzheimer's & Dementia Caregiver Protocol

The step-by-step daily care guide for every stage



STAGE 1 EARLY

STAGE 2 MIDDLE

STAGE 3 LATE

48 pages · Print-ready · Built for care professionals

Important Notice

IMPORTANT NOTICE

This guide shares practical care and communication tips for people living with Alzheimer's and other dementias. It is not medical advice. It does not diagnose, treat or cure any condition. Always follow your facility's policies and your care team's instructions. For medication, pain, infection, eating or swallowing problems, or any sudden change, report to the nurse or doctor.

ABOUT THE AUTHOR



[Author bio to be added]

What's Inside

Your step-by-step guide to confident, compassionate care.



Start Here — How to use this guide and the 7 care principles — 4

Part 1 · Find the Stage — Stage 1, 2 and 3, plus a checklist — 6

Part 2 · The Daily Routine — Routines by stage, late afternoon and nights — 13

Part 3 · Hard Situations — 16 situations: what to do, what to say, what to avoid — 21

Part 4 · Activities by Stage — The right activity for every stage — 37

Part 5 · Family and Team — Daily Summary, family phrases and self-care — 42

Closing — Pocket checklist, red flags and resources — 47

How to Use This Guide in 3 Steps

1 FIND THE STAGE



Go to pages 6 to 12. Use the checklist on page 10.

2 FIND THE SITUATION

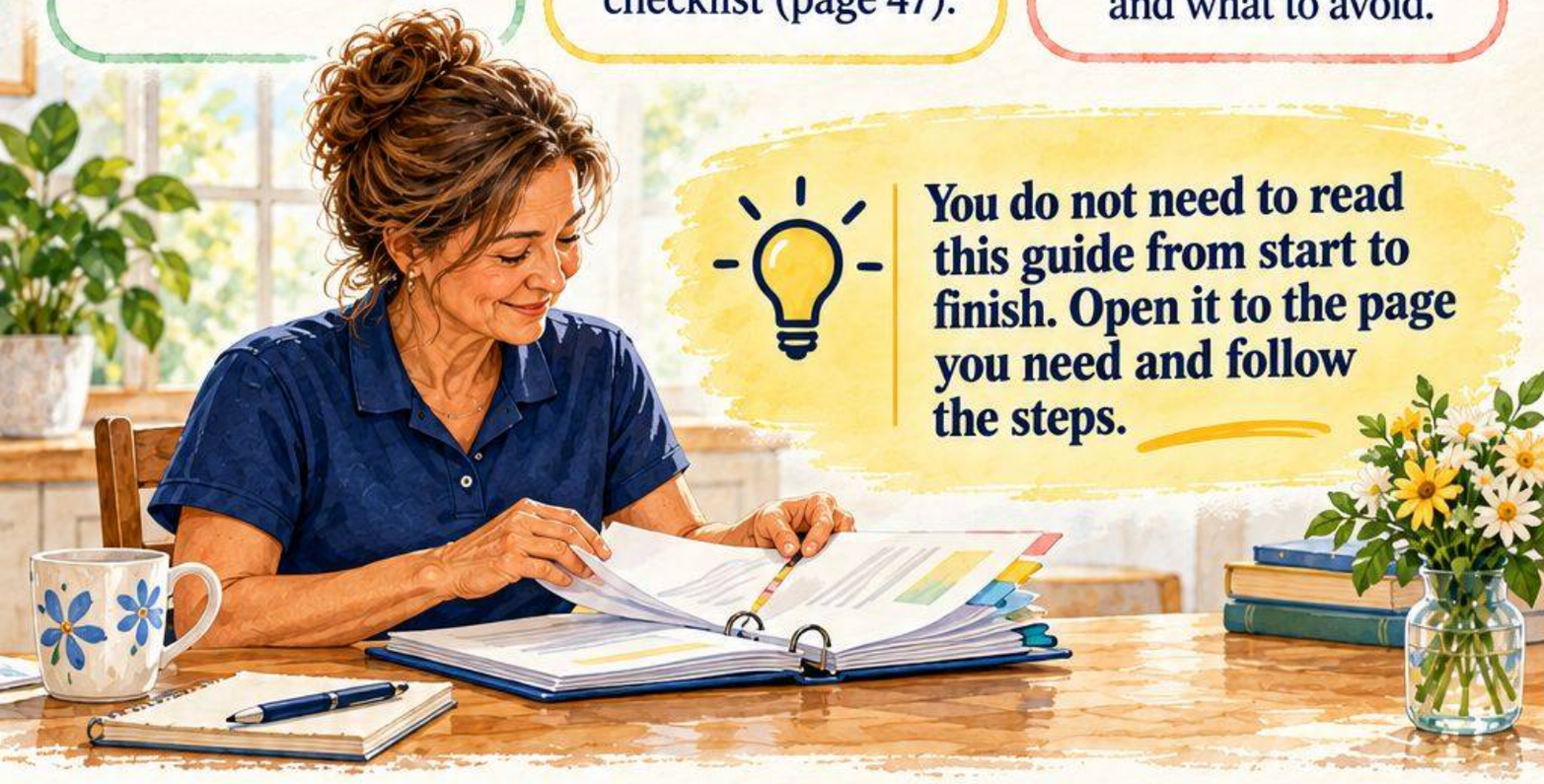


Look it up in the contents (page 3) or the pocket checklist (page 47).

3 FOLLOW THE STEPS



Every situation page shows why it happens, what to do, what to say and what to avoid.



You do not need to read this guide from start to finish. Open it to the page you need and follow the steps.

The 7 Care Principles

1. 

Calm voice, slow moves

Speak softly and move slowly. They read your tone before your words.

2. 

Don’t argue or correct

Being right does not help. Connection does.

3. 

One step at a time

Give one short instruction, then wait.

4. 

Offer simple choices

‘Blue shirt or green shirt?’ gives control back.

5. 

See the fear behind the behavior

Look for fear, pain, confusion or tiredness.

6. 

Protect dignity and privacy

Knock, cover, ask, explain. Speak to the person, not about them.

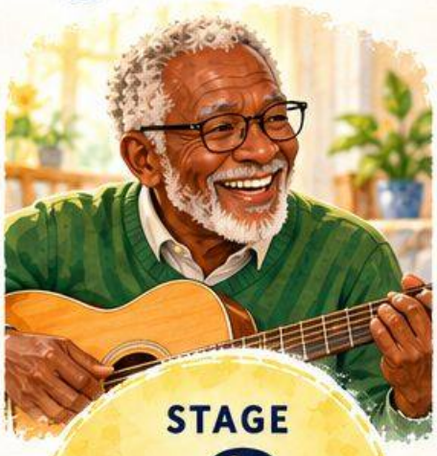
7. 

Keep a routine

Same times, same steps, same faces. Predictable is calming.

The 3 Stages at a Glance







	STAGE 1 EARLY	STAGE 2 MIDDLE	STAGE 3 LATE
WHAT THEY CAN STILL DO	Live independently, talk, make decisions, manage many daily tasks.	Do familiar tasks with cues, enjoy music and people, share feelings.	React to touch, music, tone of voice and familiar faces.
COMMON BEHAVIORS	Trouble finding words, forgetting new names, misplacing things.	Confusion about day and place, repeated questions, frustration, suspicion, sleep changes, wandering.	Little speech, limited movement, trouble swallowing, hard to show pain.
WHAT THEY NEED	Support, respect and a role in decisions.	Help with daily tasks, simple routines, patient cues.	Full care, comfort, gentle touch, a calm voice.

Stages overlap and change from day to day.
Use this as a general guide, not a strict timeline.

Stage 1 • Early (Mild)

STAGE 1 EARLY

Often still independent. May hide or cover for memory gaps.

COMMON SIGNS

- Trouble finding the right word
- Forgetting new names
- Misplacing things
- Trouble planning or organizing
- Getting lost in familiar places

HOW TO SUPPORT

- Involve them in decisions
- Protect their independence
- Help without taking over
- Use notes, calendars and routines
- Stay patient when they repeat themselves



Stage 2 · Middle (Moderate)

STAGE 2 MIDDLE

Usually the longest stage. More help is needed.

COMMON SIGNS

1. Confused about day and place
2. Frustration or withdrawal
3. Repeated questions
4. Suspicion of others
5. Changes in sleep
6. Risk of wandering
7. Bladder control problems



HOW TO SUPPORT

1. Find what they can still do
2. Break tasks into single steps
3. Keep the routine
4. Stay close in the late afternoon
5. Use cues, not corrections

Calm support builds confidence and helps them feel safe.

Stage 3 · Late (Severe)

STAGE 3 LATE

Needs care around the clock.

COMMON SIGNS

- May lose the ability to talk
- Limited movement
- Trouble swallowing
- Hard to show pain
- Sleeps more

HOW TO SUPPORT

- Comfort comes first
- Gentle touch and a soft voice
- Calm music
- Change position and watch the skin
- Talk to them even without a reply
- Watch for pain (grimacing, moaning) and report to the nurse



PART 1 · FIND THE STAGE

Find Your Senior's Stage

Check the boxes that fit today.

STAGE 1 EARLY



- ☐ Searches for words
- ☐ Forgets new names
- ☐ Misplaces objects often
- ☐ Still manages most daily tasks alone

STAGE 2 MIDDLE



- ☐ Confused about day or place
- ☐ Needs help dressing or bathing
- ☐ Repeats the same question
- ☐ Restless in the late afternoon

STAGE 3 LATE



- ☐ Speaks very little or not at all
- ☐ Needs help to eat
- ☐ Needs help to move or turn
- ☐ Reacts mostly to touch, music or voice

Most checks in one column show the closest stage.
Checks in two columns mean they are between stages.
That is normal. Use this as a general guide.

Resident Profile Sheet

Fill this out with the family. It is the base of person-centered care.

Preferred name

_____

Stage

1 · 2 · 3

Life story

_____

_____

_____

Work or profession

_____

Family (names)

_____

Favorite things

_____

_____

Favorite music

_____

_____

What calms them

_____

_____

What upsets or triggers them

_____

_____

Best times of the day

_____

_____

Daily routine notes

_____

_____

Date

_____

Filled out by

_____

Stages Overlap. Sudden Change? Call the Nurse.

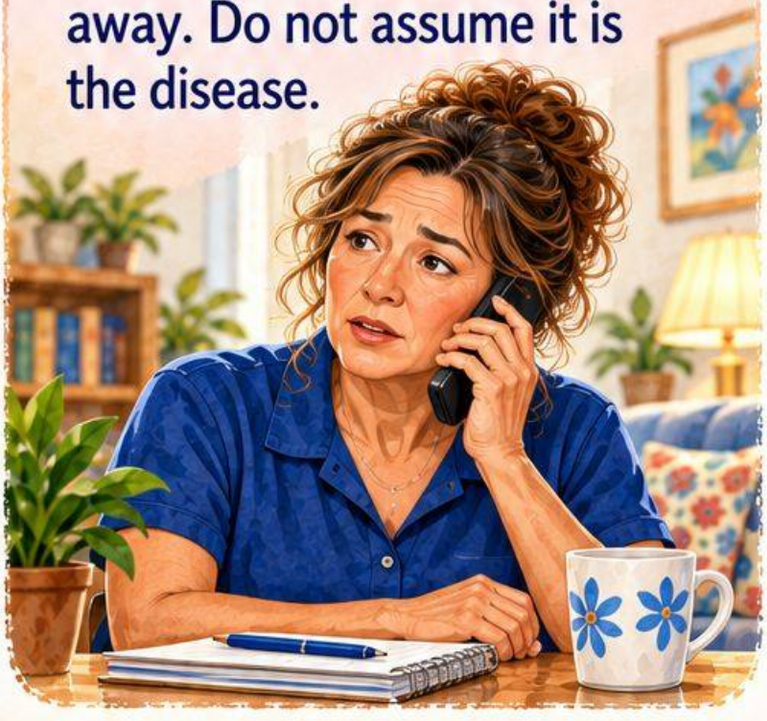
STAGES OVERLAP

The person can move between stages. Good days and hard days are normal. Signs from one stage can show up in another.



SUDDEN CHANGE? CALL THE NURSE

A change over hours or days (new confusion, extreme sleepiness, new agitation, fever, pain, new incontinence) may point to a medical problem such as an infection or delirium. Report it right away. Do not assume it is the disease.



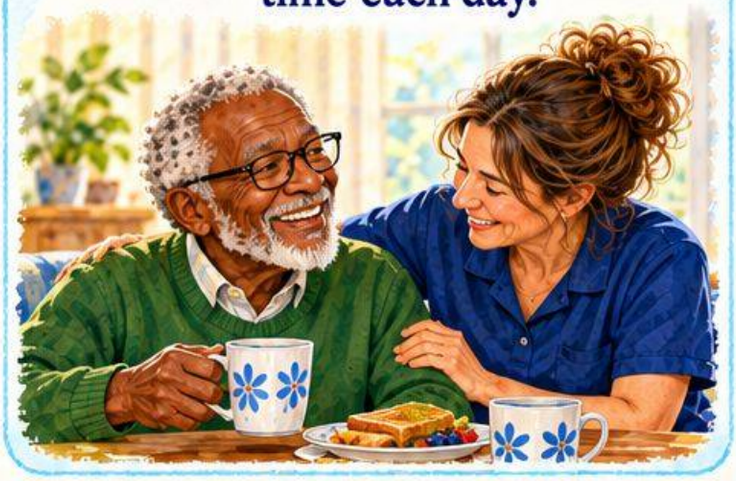
PART 2 • THE DAILY ROUTINE

Why Routine Works

Predictable days mean less fear and less agitation.

SAME TIMES

Do things at the same time each day.



SAME ORDER

Keep the same steps in the same order.



SAME FACES

Use the same caregivers when you can.



BEST TIMES

Plan baths, appointments and hard tasks for the morning or early afternoon, when they are at their best.



Daily Routine · Stage 1

STAGE 1 EARLY

7:30	Wake up and get ready (let them choose the clothes)
8:30	Breakfast together
9:30	Walk or light exercise
10:30	Mind activity (puzzle, reading, memories)
12:00	Lunch (the bigger meal)
1:00	Rest or hobby
3:00	Social time or outing
5:00	Quiet time to prepare for the evening
6:00	Light dinner
7:30	Calm evening (music, photos)
9:00	Bedtime routine

Maximum independence.

Let them lead each step.



Daily Routine · Stage 2

STAGE 2 MIDDLE

7:30		**Wake up** (offer 2 clothing choices)
8:30		**Breakfast** (one dish at a time)
9:30		**Bathing or personal care** (best time)
10:30		**Simple activity** (folding, sorting, music)
12:00		**Lunch** (the largest meal)
1:00		**Rest**
2:30		**Short walk in daylight**
3:30		**Quiet activity** with the lights on
5:00		**Calm time:** no TV, soft music
6:00		**Light dinner**
7:30		**Evening routine**
8:30		**Bedtime**

More help, simpler tasks, more breaks. Protect the late afternoon.



Daily Routine · Stage 3

STAGE 3 LATE

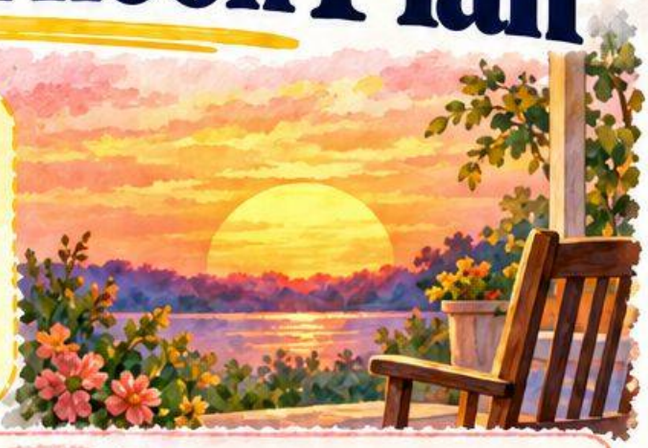
7:30	Morning: Wake gently and greet by name
8:30	Morning care: warm room, one step at a time
9:30	Breakfast with help: upright, small bites. Report coughing or choking
10:30	Sensory time: music, hand massage, soft textures
12:00	Reposition and check comfort (follow the facility plan)
1:00	Lunch with help
2:00	Rest
3:00	One-on-one time: talk, read, hold a hand
4:00	Afternoon: daylight and fresh air
5:00	Dinner with help
6:00	Calm evening: soft music, low light
7:30	Bedtime comfort

Comfort, position, gentle sensory time and one-on-one presence.



The Late-Afternoon Plan

Sundowning is confusion or agitation that gets worse in the late afternoon or evening. Many people with dementia go through it.





Lights on before dusk



Less noise and stimulation (TV, loud music, chores)



Soft music or familiar photos



Bigger lunch, lighter dinner



Daylight walk earlier in the day



No bath or appointments late in the day



Limit caffeine and sugar after lunch



Write down what happens before episodes

If it does not improve, tell the nurse or doctor.

Restless Nights and Sleep

DURING THE DAY AND EVENING



Daylight and movement during the day



A calm bedtime routine: same steps, same time



Limit long naps



No caffeine or sugar in the evening



A comfortable, safe room: nightlight, good temperature, no clutter

IF THEY ARE AWAKE AT NIGHT

1 Stay calm and keep your voice soft.



2 Check basic needs: bathroom, thirst, pain.



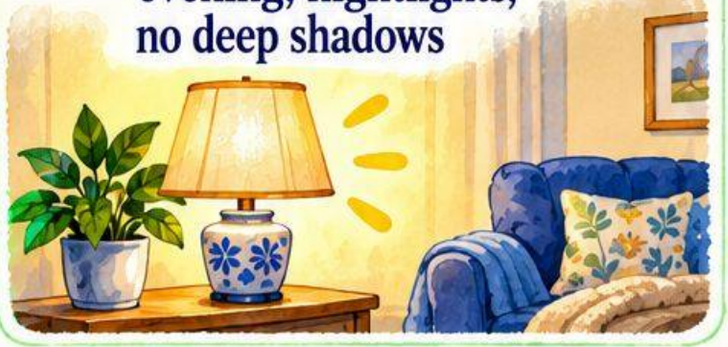
3 Offer comfort and redirect gently to bed.



Sleep problems can have medical causes. If they continue, talk to the doctor.

A Calm and Safe Environment

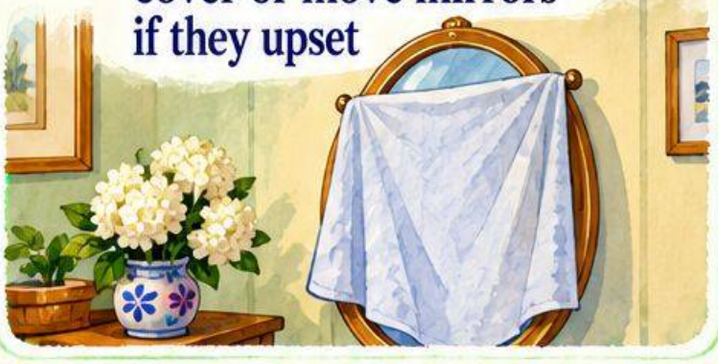
- [ ] **LIGHT:** bright in the evening, nightlights, no deep shadows



- [ ] **NOISE:** TV off when not watched, a quiet space



- [ ] **MIRRORS AND SHADOWS:** cover or move mirrors if they upset



- [ ] **FLOORS:** no loose rugs, clear paths, dry floors



- [ ] **FAMILIAR ITEMS:** photos, favorite blanket, their own clock



- [ ] **SIMPLE SIGNS:** picture signs on the bathroom and bedroom doors



- [ ] **EXITS:** door alarms or chimes, as the facility policy says



- [ ] **BATHROOM:** grab bars, nonslip mat, clear labels



New Resident: The First 14 Days

PHASE 1
DAY 1 TO 2



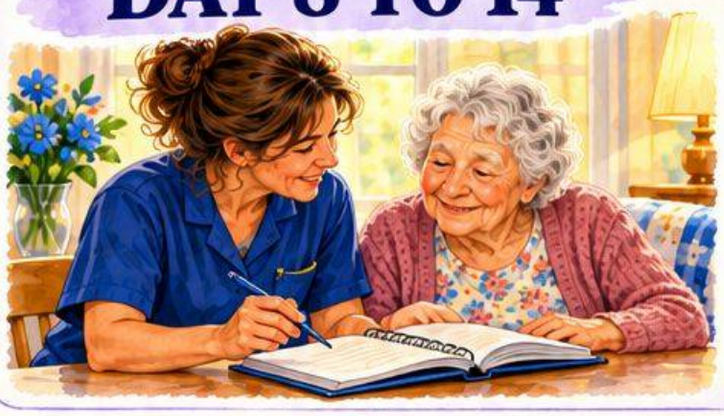
- Introduce yourself every time.
- Keep familiar objects close.
- Fill out the profile sheet with the family.
- Keep visits short and calm.

PHASE 2
DAY 3 TO 7



- Keep the same routine and the same caregivers when you can.
- Watch what calms them and what triggers them.
- Start activities slowly.

PHASE 3
DAY 8 TO 14



- Adjust the routine with what you learned.
- Update the profile sheet.
- Share your notes with the team and the family.

PART 3 • HARD SITUATIONS

Late-Afternoon Agitation

STAGE 2 • MIDDLE

WHY IT HAPPENS



They are tired after a long day. Low light and shadows, too much noise and stress around them add up. They are not being difficult.



WHAT TO DO

1.  Lower your voice.
2.  Reduce the noise. Turn off the TV.
3.  Offer a simple task for their hands.
4.  Sit beside them.
5.  Play calm music or show photos.

SAY THIS

> *"I'm here. You're safe. Would you like to help me fold these towels?"*

AVOID

- ✕ Arguing or correcting
- ✕ Holding or restraining
- ✕ Large groups and crowded rooms
- ✕ Baths or appointments at this time

BY STAGE

STAGE 1 • EARLY
May feel anxious. Offer a calm task and a quiet space.

STAGE 2 • MIDDLE
Peak time. Follow the late-afternoon plan on page 17.

STAGE 3 • LATE
May moan or fidget. Soft music, gentle touch, a comfortable position.

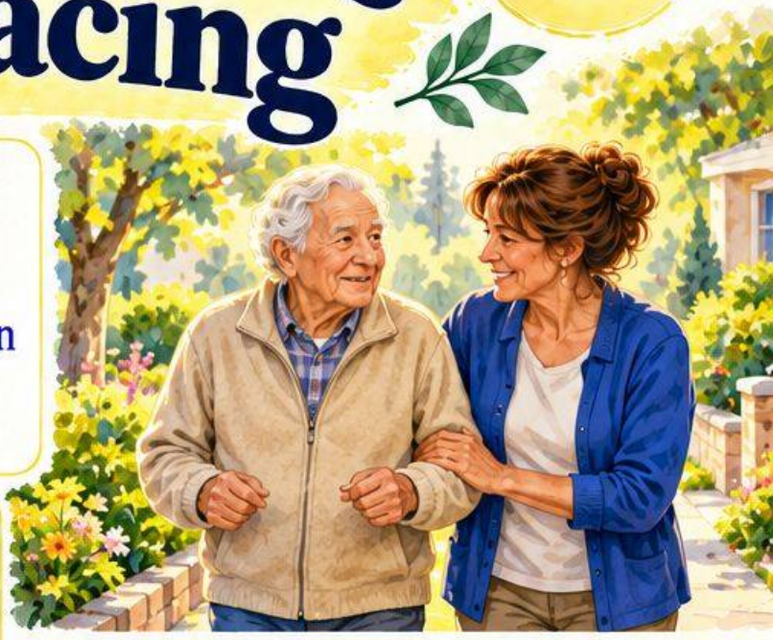
ASK FOR HELP IF

It happens almost every day, gets worse, or comes with a sudden change. Tell the nurse or doctor.

PART 3 • HARD SITUATIONS

Restlessness and Pacing

STAGE 2 • MIDDLE



WHY IT HAPPENS



Boredom, unmet needs, extra energy or anxiety. Pacing is often a need, not a problem.

WHAT TO DO

1.  Check needs first: bathroom, thirst, hunger, pain, temperature.
2.  Walk with them on a safe route.
3.  Offer a rhythmic task: folding, sorting, stirring.
4.  Let them sit and rest when they are ready.
5.  Keep paths clear and shoes safe.

SAY THIS

> *"Let's take a walk together. I'll come with you."*

AVOID

- Stopping them by force
- Saying 'Sit down!' or 'Stop walking!'
- Leaving them alone while they pace

BY STAGE

STAGE 1 • EARLY

May be worried. Talk about what is on their mind.

STAGE 2 • MIDDLE

Most common. Plan walks and tasks.

STAGE 3 • LATE

May fidget in bed or chair. Change position, offer a soft touch.

ASK FOR HELP IF

The pacing is new or sudden, or comes with pain, fever or falls.

PART 3 • HARD SITUATIONS

Aggression

STAGE 2 • MIDDLE



WHY IT HAPPENS

Fear, pain, feeling rushed or cornered, noise, or not understanding what is happening. It is not personal.

WHAT TO DO

1. Safety first. Give them space and keep a clear way out for you.
2. Stay at their eye level, a little to the side.
3. Stay calm. Speak low and slow.
4. Remove the trigger: noise, crowd, task.
5. Step away and get help as your facility policy says. Report afterward.

SAY THIS

"I'm sorry you're upset. I'll give you some space."

AVOID

- Shouting or arguing
- Grabbing or holding them
- Standing over them or blocking the way

BY STAGE

STAGE 1 • EARLY
Often frustration. Give back control and choices.

STAGE 2 • MIDDLE
Look for the trigger: pain, rush, noise.

STAGE 3 • LATE
May strike out from pain or fear. Check for pain and offer comfort.

ASK FOR HELP IF

Anyone is hurt, it keeps happening, or it is a new behavior. Report and record it. Follow your facility policy.

Refusing to Bathe

STAGE 2 • MIDDLE | STAGE 3 • LATE



WHY IT HAPPENS

Fear, loss of privacy, feeling cold, the noise of the water, or not understanding what is happening. They are not being difficult.

SAY THIS

"Which towel would you like, the blue or the pink?"

WHAT TO DO

1. Do not argue or insist.
2. Explain one step at a time.
3. **Let them choose between two options, if they can still respond. For example, towel A or towel B.**



4. Protect their privacy. Cover them with a towel.
5. Warm the room and use a gentle touch.

AVOID

- Forcing or arguing
- Rushing
- Exposing the person
- Bathing late in the day

If they refuse, stop and try later. Bathing does not have to happen every day. A sponge bath can do.

BY STAGE

Stage 1 Early	Usually bathes with reminders. Leave supplies ready and give privacy.
Stage 2 Middle	Help step by step. Use choices and a warm room.
Stage 3 Late	Full help. Warm room, gentle touch, talk through each step.

ASK FOR HELP IF

You see skin problems or pain, or the refusal lasts several days. Tell the nurse.

PART 3 • HARD SITUATIONS

Refusing Food and Fluids

STAGE 2 • MIDDLE

STAGE 3 • LATE

WHY IT HAPPENS



Not feeling hungry, trouble chewing or swallowing, too much on the plate, noise, not recognizing the food, or pain.



WHAT TO DO

1.  Quiet table. TV off.
2.  One dish at a time, small portions.
3.  Finger foods and plates with contrasting colors.
4.  Offer drinks often through the day.
5.  Sit with them and eat together.

SAY THIS

"Here is a little soup. Would you like to try some?"

AVOID

- Forcing food
- Rushing
- Talking while they chew
- Offering many things at once

BY STAGE

STAGE 1 • EARLY

May forget meals. Keep a regular meal schedule.

STAGE 2 • MIDDLE

Simplify: one dish and clear cues.

STAGE 3 • LATE

Upright position, small bites, patience. Report coughing or choking right away.

ASK FOR HELP IF

They cough or choke while eating, lose weight, have mouth pain, or refuse fluids for a day. Tell the nurse or doctor.

PART 3 • HARD SITUATIONS

Refusing Medication

STAGE 2 • MIDDLE

STAGE 3 • LATE

WHY IT HAPPENS



Fear, not remembering why they need it, the taste, pills that are hard to swallow, or feeling pressured.



WHAT TO DO

1. Follow your facility policy and the doctor's orders.
2. Pick a calm moment.
3. Explain in simple words, one step at a time.
4. If they refuse, try again later as the policy allows.
5. Record it and tell the nurse.

SAY THIS

"This is your morning pill. It helps you feel good. Here is some water."

AVOID

- Hiding or crushing medication without authorization from the doctor and the facility
- Arguing or forcing
- Lies that cause distress

BY STAGE

STAGE 1 • EARLY

May accept with a pill box and reminders.

STAGE 2 • MIDDLE

Calm timing and simpile words.

STAGE 3 • LATE

The nurse decides the method. Keep them comfortable.

ASK FOR HELP IF

Always report refusals to the nurse. Never change or skip medication on your own.

PART 3 • HARD SITUATIONS

Personal Care and Changing, with Dignity

STAGE 2 • MIDDLE

STAGE 3 • LATE



WHY IT HAPPENS

Losing independence feels scary and embarrassing. They may not recognize the need or understand what you are doing.

WHAT TO DO

1. Privacy first. Close the door and cover them with a towel.
2. Keep a bathroom routine, every 2 to 3 hours as the care plan allows.
3. Choose easy clothes: elastic waists, large buttons, velcro.
4. Tell them each step before you do it.
5. Stay calm and kind. Never show disgust.

SAY THIS

"Let's get you comfortable. I'll help with your pants now."

AVOID

- Rushing
- Talking about them as if they were not there
- Scolding after accidents
- Exposing the person

BY STAGE

STAGE 1 • EARLY
Remind, then step out.

STAGE 2 • MIDDLE
Help with each step.

STAGE 3 • LATE
Full help, gentle handling, skin care. At night use soft light and a calm voice.

ASK FOR HELP IF

You see redness or sores, or signs of a urinary infection (new confusion, burning, strong smell), or pain. Tell the nurse.

PART 3 • HARD SITUATIONS

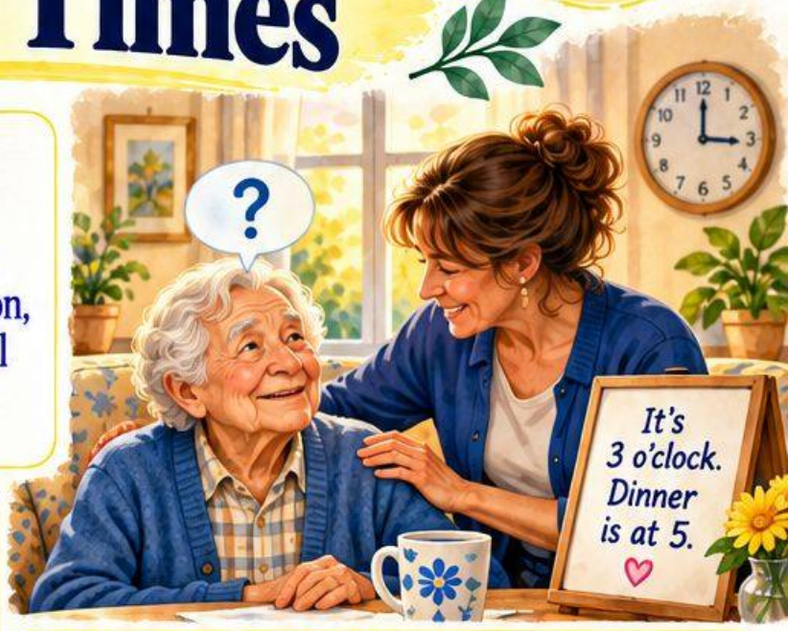
The Same Question, Twenty Times

STAGE 2 • MIDDLE

WHY IT HAPPENS



They cannot store the answer. The feeling behind the question, like worry or insecurity, is still there.



WHAT TO DO

1. Answer calmly, as if it were the first time.
2. Listen for the feeling behind it.
3. Write the answer on a card or board where they can see it.
4. Redirect with a simple task or a favorite topic.



SAY THIS

"It's 3 o'clock. Dinner is at 5. You're safe."

AVOID

- Saying 'I already told you.'
- Sighing or showing frustration
- Quizzing them: "Do you remember?"

BY STAGE

STAGE 1 • EARLY
Use a calendar and notes.

STAGE 2 • MIDDLE
Cards, a board and calm repeats.

STAGE 3 • LATE
Little speech. Answer the mood with touch and tone.

ASK FOR HELP IF

The repeating comes with new distress or a sudden change.

PART 3 • HARD SITUATIONS

Calling for Someone Who Has Passed Away, or Asking to Go Home

STAGE 2 • MIDDLE

WHY IT HAPPENS



They are living in an earlier time. "Home' often means safety and comfort.



WHAT TO DO

1. Acknowledge the feeling. Do not argue.
2. Do not force reality or repeat the news of the death.
3. Ask about the person: "Tell me about her."
4. Look at photos together.
5. Redirect gently to a calm activity.

SAY THIS

"You miss her. Tell me about her."

AVOID

- Saying "She died years ago.'
- Arguing about where they live
- Lies that cause pain

BY STAGE

STAGE 1 • EARLY
May grieve with full awareness. Listen.

STAGE 2 • MIDDLE
Most common. Follow the feeling.

STAGE 3 • LATE
Little speech. Soft voice, touch, familiar music.

ASK FOR HELP IF

The sadness is intense or lasts, they stop eating or sleeping, or they talk about harming themselves. Tell the nurse or doctor right away.

PART 3 • HARD SITUATIONS

Not Recognizing Family or Caregivers

STAGE 2 • MIDDLE

STAGE 3 • LATE



WHY IT HAPPENS

Memory for faces and names fades. It is the disease, not a lack of love.

WHAT TO DO

1. Do not take it personally.
2. Introduce yourself every time with your name and relationship.
3. Skip the quiz. Do not ask "Do you know who I am?'
4. Use a name tag and a family photo.
5. Connect through feelings, voice and touch.

SAY THIS

"Hi Mom, I'm Anna, your daughter. I'm so happy to see you."

AVOID

- Saying 'Don't you remember me?'
- Correcting or arguing
- Pulling away if they pull back

FOR FAMILY

They are still the same person inside. Your presence matters, even if they do not know your name.

BY STAGE

STAGE 1 • EARLY
May forget recent names. Gently offer them.

STAGE 2 • MIDDLE
Introduce yourself every time.

STAGE 3 • LATE
May not recognize anyone. Connect through voice and touch.

ASK FOR HELP IF

Share new changes with the team and the family.

Accusations and Suspicion

STAGE 2 • MIDDLE



WHY IT HAPPENS

They misplace things and cannot remember where, so the mind looks for someone to blame. Fear and feeling out of control.

WHAT TO DO

1. Do not argue or defend yourself.
2. Reassure them: "I'll help you look."
3. Search together. Check usual hiding spots: purse, drawers, trash.
4. Keep duplicates of loved items: glasses, keys, wallet.
5. Respond to the feeling, not the accusation.

SAY THIS

"I know it's upsetting. Let's look for it together."

AVOID

- Saying "I didn't take it!"
- Arguing about facts
- Taking it personally

BY STAGE

Stage 1 Early	May suspect after losing things. Give items a "home" and use notes.
Stage 2 Middle	Most common. Search together.
Stage 3 Late	Rare. Offer comfort.

ASK FOR HELP IF

The suspicion keeps growing or the fear gets worse. Report to the nurse or your supervisor.

Wanting to Leave, Wandering

STAGE 2 • MIDDLE



WHY IT HAPPENS

They may be looking for someone or something, feel restless or lost, or follow an old routine, like 'going to work'.

WHAT TO DO

1. Safety first: ID on them and safe exits, as your facility policy says.
2. Watch the high-risk times, especially the late afternoon.
3. Find the purpose: "Where are you going?"
4. Walk with them, then redirect with a task or a snack.
5. Know the emergency plan. If they leave, tell your supervisor right away.

SAY THIS

"Let's go together. First, would you like some tea?"

AVOID

- Blocking or grabbing them
- Arguing
- Leaving them alone near exits

BY STAGE

Stage 1 Early	May get lost in familiar places. Use ID and check in.
Stage 2 Middle	Highest risk. Supervise and plan walks.
Stage 3 Late	Rarely wanders. Falls are the risk when they try to move.

ASK FOR HELP IF They leave, or almost leave. Report it right away and follow your emergency plan.

Refusing to Join Activities

STAGE 2 • MIDDLE



WHY IT HAPPENS

They fear failing, do not understand what is asked, feel tested, or the activity feels childish.

WHAT TO DO

1. Do not insist. Come back later.
2. Start the activity yourself, next to them.
3. Use familiar adult roles: folding, sorting, setting the table.
4. Keep it short, with guaranteed success. No tests.
5. Adapt it to their stage.

SAY THIS

"I could use your help with this. Would you like to try?"

AVOID

- Pressuring or scolding
- Childish materials
- Questions with a "right answer"

BY STAGE

Stage 1 Early	Puzzles, memories, cooking.
Stage 2 Middle	One-step tasks and music.
Stage 3 Late	Sensory activities and music.

ASK FOR HELP IF

They refuse everything for several days.
It may be pain, mood or health. Tell the nurse.
See Part 4, pages 37 to 41.

Apathy and Withdrawal

STAGE 2 • MIDDLE

STAGE 3 • LATE



WHY IT HAPPENS

Loss of interest is part of the disease. It can also come from depression, pain, or poor hearing or vision.

WHAT TO DO

1. Bring back old interests: gardening, music, cards.
2. Offer one-on-one time.
3. Add daylight, fresh air, a change of place.
4. Give small tasks with a purpose.
5. Use sensory stimulation: music, scents, textures.

SAY THIS

"I'd love some company. Will you sit with me for a while?"

AVOID

- Leaving them alone for long
- Forcing group activities
- Assuming nothing helps

BY STAGE

Stage 1 Early	May feel low. Encourage routines and connection.
Stage 2 Middle	Gentle invitations.
Stage 3 Late	Presence, touch and music.

ASK FOR HELP IF

Check vision, hearing, pain and mood. Report to the nurse or doctor.

Fear, Hallucinations and Confusion

STAGE 2 • MIDDLE STAGE 3 • LATE



WHY IT HAPPENS

They may misread shadows, reflections or noise. Poor vision or hearing, tiredness, and sometimes illness add to it.

WHAT TO DO

1. Check the light, shadows and mirrors.
2. Check glasses and hearing aids.
3. Do not argue about what they see.
4. Reassure them: “You’re safe. I’m here.”
5. Change the room or the scene.

SAY THIS

“I don’t see it, but I can tell it’s frightening. I’m here with you.”

AVOID

- Saying “There’s nothing there!”
- Laughing
- Leaving them alone

BY STAGE

Stage 1 Early	Rare. Check eyesight.
Stage 2 Middle	More common. Adjust the environment.
Stage 3 Late	Calm, soft voice and touch.

ASK FOR HELP IF

It starts suddenly. Tell the doctor right away. It can be delirium, a medical emergency.

Undressing and Inappropriate Behavior

STAGE 2 • MIDDLE | STAGE 3 • LATE



WHY IT HAPPENS

Feeling hot, needing the bathroom, uncomfortable clothes, confusion, or not remembering what is appropriate. It is the disease, not the person.

WHAT TO DO

1. Stay calm and protect their privacy.
2. Check needs: too warm, bathroom, uncomfortable clothes.
3. Offer soft, simple clothes that are easy to manage.
4. Redirect to another activity.
5. Never shame them.

SAY THIS

"Let's get you comfortable. Here, put this robe on."

AVOID

- Scolding or laughing
- Exposing them in front of others
- Punishing

BY STAGE

Stage 1 Early	Rare. Talk privately.
Stage 2 Middle	Look for the cause.
Stage 3 Late	Comfort and privacy.

ASK FOR HELP IF

The behavior is new or sudden. Report it to the nurse or doctor.

How to Choose the Right Activity

The best activity feels like a normal day, not a test.



SHORT:
10 to 15 minutes



FAMILIAR:
things they have done before



WITH AN ADULT ROLE:
helping, not playing



GUARANTEED SUCCESS:
no way to get it wrong



NO TESTS OR QUIZZES



THEIR PACE:
stop when they are done



Less is more.

One or two simple activities are better than a long list.



TIP

Start the activity yourself and invite them in.

Stage 1 Activities

STAGE 1 • EARLY



Reminiscence: photo albums and life stories



Word games and puzzles in large print



Crafts: painting, cards



Cooking or baking together



Light exercise: walking, chair exercises



Conversation about their life story



FROM YOUR BRIGHTER DAYS KIT



Finish the Saying



Name Five!



Shape Sudoku



Road Trip USA



Golden Age Movie Trivia

Stage 2 Activities

STAGE 2 • MIDDLE





Simple tasks: folding towels, sorting buttons or socks, arranging flowers, wiping the table



Music and singing



Sensory activities: hand lotion, scents, textures



Short walks



One-step games: matching, dominoes

FROM YOUR BRIGHTER DAYS KIT



Seated Hand Dance



Name That Tune



The Senses Box



Balloon Volleyball



Color Dominoes

Start the task yourself and invite them in.

Stage 3 Activities

STAGE 3 · LATE



Calm music



Hand massage



Gentle touch



Family photos



Sounds of nature



Presence: sit, hold a hand, talk



FROM YOUR BRIGHTER DAYS KIT



Hand Spa Day



Homemade Aromatherapy



Guided Meditation



The Senses Box



Name That Tune



Focus on comfort and connection. There are no wrong results.

Group Activities with Mixed Stages



Many groups include people in different stages.
Choose activities with more than one way to take part.

STAGE 1 · EARLY

Leads, helps explain, keeps score.

STAGE 2 · MIDDLE

Does the simple steps: sorting, matching, passing.

STAGE 3 · LATE

Takes part by watching, listening, music and touch.

GOOD MIXED-STAGE IDEAS

- Group singing
- Balloon Volleyball
- Name That Tune
- Flower arranging
- Photo memories

KIT MAP

Stage 1	Stage 2	Stage 3
Finish the Saying, Shape Sudoku	Color Dominoes, The Senses Box	Hand Spa Day, Guided Meditation

Talking with Families

Families are part of the care team. Honest, kind updates build trust.



1. Be honest and kind.



2. Share good moments too, not only problems.



3. Use simple words, no jargon.



4. Listen first. Families are often grieving.



5. Share what you see. Leave diagnosis and treatment to the nurse and doctor.



A GOOD UPDATE SOUNDS LIKE

"Today she enjoyed the music and ate most of her lunch. She was restless around 4. We put on soft music and she settled."

Daily Summary for the Family

Fill in at the end of the shift. Share with the family and the team.

Name _____

Date _____

MOOD today



SLEEP

☐ good ☐ fair ☐ poor

MEALS and FLUIDS ate: ☐ all ☐ half ☐ little

_____

ACTIVITIES today

1. _____
2. _____

ANYTHING TO REPORT to the nurse or doctor

_____

_____

Filled out by _____

SUGGESTED PHRASES FOR THE FAMILY

"Today she enjoyed the music and ate well."

"He was restless around 4 pm, so we dimmed the lights and he settled."

"She had a calm afternoon and asked about you."



12 Phrases to Share with Families

Simple words that help families feel supported.

WHEN THEY ARE WORRIED



- We are taking good care of her.
- It is normal to feel worried.
- We will keep you updated.

WHEN THEY ARE SAD



- I can see how much you love him.
- Your visits mean a lot.
- You are not alone in this.

WHEN THEY ASK 'WHY DOESN'T SHE KNOW ME?'



- It is the disease, not a lack of love.
- She still feels your love.
- Your voice and touch still comfort her.

WHEN THEY WANT TO HELP



- Bring photos and music she loves.
- Visit at her best time of day.
- Sit with her. Presence matters.

Guiding Family Visits

BEFORE THE VISIT



Pick their best time of day.



Keep it short, 30 minutes is plenty.



Choose a quiet place.



Prepare the family: stage, what to expect.

DURING THE VISIT



Introduce yourself every time.



Bring photos, music or a simple activity.



Do not quiz them.



Say goodbye calmly and do not make a big scene.



TELL THE TEAM

Share how the visit went. Report any change to the nurse.

Take Care of Yourself

You cannot pour from an empty cup.
Caregiver stress is real.



REST:
Take your breaks.



TALK:
Share with a coworker or supervisor.



MOVE:
A short walk helps.



BREATHE:
Three slow breaths before a hard moment.



ASK:
Ask for help early.



LET GO:
It is the disease, not your fault.



Feeling exhausted, hopeless or burned out?

Talk to your supervisor or a health professional.
Support is available.

Pocket Checklist

Print, cut and keep it with you.

- [ ]  Calm voice, slow moves
- [ ]  Introduce yourself
- [ ]  One step at a time
- [ ]  Offer two choices
- [ ]  Do not argue or correct
- [ ]  Look for the fear behind it
- [ ]  Check needs: bathroom, thirst, hunger, pain
- [ ]  Protect privacy and dignity
- [ ]  Keep the routine
- [ ]  Late afternoon: lights on, less noise
- [ ]  Record what happened
- [ ]  Report to the nurse or doctor

Red Flags, Resources and Sources

CALL THE NURSE OR DOCTOR RIGHT AWAY

- Sudden confusion or change over hours or days
- Fever, pain or signs of infection
- Falls or injury
- Choking, coughing when eating, or refusing all fluids
- New hallucinations or extreme agitation
- Talk of self-harm

IN AN EMERGENCY CALL 911.

RESOURCES

Alzheimer's Association
24/7 Helpline: 1-800-272-3900
alz.org

National Institute on Aging:
nia.nih.gov

SOURCES

Alzheimer's Association (alz.org)

National Institute on Aging (nia.nih.gov)

Thank you for the care you give every day.